

**Performance Work Statement for Military Sealift Command's Hew Hire Medical Evaluation
Support Services**

Background. Commander, Military Sealift Command (COMSC), in his role as single Department of Defense (DoD) manager for sea transportation, operates a worldwide command and a Government-owned fleet of approximately 116 ships that provides strategic and operational sealift to meet national defense objectives. To properly crew these vessels medically fit Civilian Mariners (CIVMAR) are required, and this starts with the recruitment and screening of Applicants. Due to the remoteness with which Military Sealift Command (MSC) vessels operate, medical risks must be carefully managed by MSC Force Medical and why CIVMAR applicants' medical history must be reviewed.

Description. The Contractor shall provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, other items, and non-personal services necessary to perform MSC's Hew Hire Medical Evaluation Support Services as defined in this Performance Work Statement (PWS) except for those items specified as government furnished property and services. The Contractor shall perform to the standards in this contract.

Objective. The objective of this contract is to provide necessary support to MSC's Force Medical to accomplish medical suitability screening of CIVMAR applicants.

Non-Personal Services. Non-personal Services: This contract may not be used to provide personal services (reference Department of Defense Handbook of Contract Function Checklists for Service Acquisition). Government personnel may not perform any supervisory functions for Contractor personnel. All individual Contractor employee assignments and daily work direction shall be given by the applicable Contractor employee's supervisor. Government personnel cannot supervise Contractor employees, stipulate Contractor duty hours (except for that directed in the contract), require Contractor employees to report to Government personnel, maintain Contractor personnel records/timecards, approve leave for Contractor personnel, approve bonuses for Contractor personnel, or develop duty rosters including name of Contractors. Notwithstanding any other provision of this contract, the Contractor shall always be responsible for the supervision of its employees in the performance of the services required. If the Contractor believes any government action or communication has been given that would create personal services relationship between the Government and any Contractor employee, the Contractor shall promptly notify the Contracting Officer.

Specific Tasks. The Contractor shall provide services for the Government's medical suitability screening of CIVMAR applicants. The Contractor shall be prepared to remotely provide these services in all areas of the country where the Government has requirements to screen CIVMAR applicants. Specific task descriptions and instructions, sufficient in detail to allow for Contractor performance, shall be provided within this contract.

The Government will provide full and detailed medical standards and guidance pursuant to the CIVMAR medical qualification standards and be available for consultation for cases that are not clearly defined and keep the Contractor updated on changes in standards and/or guidance.

1. **Period of Performance:** A one-year base period, four one-year option periods, and an option to extend services for up to an additional six-months in accordance with FAR 52.217-8. The estimated Period of Performance reads as follows:

Period	Date of Commencement	Date of Completion
Base Period	07 June 2026	06 June 2027
Option Period One	07 June 2027	06 June 2028
Option Period Two	07 June 2028	06 June 2029
Option Period Three	07 June 2029	06 June 2030

Option Period Four	07 June 2030	06 June 2031
FAR 52.217-8, Six-Month Option	07 June 2031	06 December 2031

2. General Information:

- 2.1. Type of Contract: The government will award a Firm Fixed Price Contract.
- 2.2. Place of Performance: The place of performance shall be the Contractor's facility.
- 2.3. Recognized Holidays: The Contractor shall follow all federal government holidays. Federally recognized holidays are defined on the Office of Personnel Management website: <https://www.opm.gov/policy-data-oversight/pay-leave/federal-holidays/>.
- 2.4. Hours of Operations: The Contractor is responsible for conducting business, between the hours of 0700-1700 Monday thru Friday, except Federal holidays or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings. The Contractor must always maintain an adequate workforce for the uninterrupted performance of all tasks defined within this PWS when the Government facility is not closed for the above reasons. When hiring personnel, the Contractor shall keep in mind that the stability and continuity of the workforce are essential.
- 2.5. Quality Assurance: The Government shall evaluate the Contractor's performance under this contract in accordance with the Performance Requirement Summary (PRS). It defines how the performance standards will be applied, the frequency of surveillance, and the minimum acceptable defect rate(s).
- 2.6. Security Requirements: This contract requires access to a significant amount of Personally Identifiable Information (PII), and Protected Health information (PHI), both categorized under Controlled Unclassified Information (CUI), which is information that does not meet the criteria for classification but must still be protected.
 - 2.6.1. Facility Security Officer (FSO): The contractor will have a U.S. citizen employee who is cleared as part of their facility clearance (FCL) to be the FSO. The FSO will be responsible for obtaining fingerprinting support/services for background investigation processing for their contractors/subcontractors.
 - 2.6.2. Personnel Security Requirements: Each individual must possess and must possess and maintain at a minimum, a Secret clearance, or a favorably adjudicated Tier 3 investigation annotated in the Defense Information System for Security (DISS) or successor system, throughout the duration of the contract performance.
 - 2.6.2.1. Security Clearance: Personnel require access to information that may be deemed sensitive and control marked a CUI.
 - 2.6.2.2. DoD personnel and DoD contractors have an affirmative responsibility to protect an individual's privacy when maintaining his/her PII or government shared PII/PHI.
 - 2.6.2.3. DoD personnel and DoD contractors conduct themselves consistent with the established rules of conduct in §310.8, so that records maintained in a system of records will only be maintained as authorized by 5 U.S.C. 552a and this part.
 - 2.6.2.4. Limit the availability of records containing PII to DoD personnel and DoD contractors who have a need to know in order to perform their duties.
 - 2.6.3. Contractor personnel shall adhere to the below directives during the performance of this contract.
 - 2.6.3.1. SECNAVINST 5510.30 Series titled, "Department of the Navy Personnel Security Program Instruction"

- 2.6.3.2. SECNAVINST 5510.36 Series titled, "Department of the Navy (DON) Information Security Program (ISP) Instruction"
- 2.6.3.3. 32 CFR, Part 117 (NISPOM), National Industrial Security Program Operating Manual
- 2.6.3.4. DoD Manual 6025.18 Series titled, "Implementation of The Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule in Dod Health Care Programs"
- 2.6.3.5. SECNAVINST 5211.5E Series titles "Department of the Navy Privacy Program:

3. References and Acronyms

3.1. References

- 3.1.1. CIVMAR MEDICAL AND PHYSICAL STANDARDS - N02.02H.6110.2-Q
- 3.1.2. Force Medical Waiver Guide
- 3.1.3. OPNAVINST 3591.1. SERIES
- 3.1.4. Navy and Marine Corps Public Health Center Technical Manual NMCPHC-TM OM 6260 Medical Surveillance Procedures Manual and Medical Matrix

3.2. Acronyms

AAOHN	American Association of Occupational Health Nurses
ABPM	American Board of Preventative Medicine
ASR	Applicant Suitability Report
CIVMAR	Civilian Mariner
CMC	Chronic Medical Condition
COMSC	Commander, Military Sealift Command
COR	Contracting Officer's Representative
CSA	Cognizant Security Agency
CSC	CIVMAR Support Center
DO	Doctor of Osteopathy
DOD	Department of Defense
EEO	Equal Employment Office
FAR	Federal Acquisition Regulation resulting from the Revolutionary FAR Overhaul, found at website: https://www.acquisition.gov/far-overhaul , implemented by DoD

Class Deviations (CDs):

2026-O0002	2026-O0003	2026-O0004	2026-O0005
2026-O0006	2026-O0007	2026-O0008	2026-O0009
2026-O0010	2026-O0011	2026-O0012	2026-O0013
2026-O0014	2026-O0015	2026-O0016	2026-O0017
2026-O0018	2026-O0019	2026-O0020	2026-O0021
2026-O0022	2026-O0023	2026-O0024	2026-O0025
2026-O0026	2026-O0027	2026-O0028	2026-O0029
2026-O0030	2026-O0031	2026-O0032	2026-O0034
2026-O0035	2026-O0036	2026-O0037	2026-O0038
2026-O0039	2026-O0040	2026-O0041	2026-O0042
2026-O0043	2026-O0045	2026-O0046	2026-O0047
2026-O0048	2026-O0049	2026-O0050	

FCL	Facility Clearence
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FSO	Facility Security Officer
HIPAA	Health Insurance Portability and Accountability Act
JFTR	DoD Joint Travel Regulations
KO	Contracting Officer
LTBI	Latent Tuberculosis Infection
MD	Medical Doctor
MSC	Military Sealift Command
MSC SQ	Military Sealift Command Screening Questionnaire (also called Summary Questionnaire)
MSP	Medical Surveillance Program
MYMSC	MSC's HR Database
N1	Total Force Management
NCD	Not Conditionally Disqualifying
NEO	New Employee Orientation
NFFH	Not Fit for Hire
NP	Nurse Practitioner
NPQ	Not Physically Qualified
NPQ/WG	Not Physically Qualified/Waiver Granted
NPQ/WR	Not Physically Qualified/Waiver Requested
OHS	Occupational Health Services contract
PA	Physician Assistant
PDF	Portable Document Format
PHI	Protected Health Information
PII	Personally Identifying Information
PNFFH	Permanently Not Fit for Hire
PQ	Physically Qualified
PRS	Performance Requirement Summary
PWS	Performance Work Statement
QFT/QFT-G	QuantiFERON Gold
RA	Reasonable Accommodation
RN	Registered Nurse
SQ	Screening Questionnaire
TB	Tuberculosis
TB BCG	Tuberculosis Bacille Calmette-Guérin Vaccine
USCG	United States Coast Guard
USCG 719K	United States Coast Guard Medical Exam Form Number 719K -NON-ENTRY LEVEL
USCG 719KE	United States Coast Guard Medical Exam Form Number 719KE - ENTRY LEVEL

4. Tasks: The contractor shall perform the following tasks: All days referenced are calendar days except for holidays. All weekly reports are due on Fridays or the next duty day. All monthly reports are due 5 business days after the close of each month. The contractor shall perform all aspects of this task with qualified personnel and have the capability of surging for short periods without reduced support.

- 4.1. Applicant Suitability Report (ASR): The Contractor shall conduct a thorough review of an applicant's medical history, assess whether the applicant meets MSC medical standards based upon the review, and generate an ASR. The contractor is required to have a supervising Medical Director (MD or DO) but is otherwise free to utilize their preferred mix of providers (MD or DO), mid-levels (PA or NP), nursing (RN), and administrative staff to accomplish the task. This new ASR process and MSC Force Medical's current New Hire process is intended to run simultaneously while the Contractor builds its process and establishes a satisfactory track record after which MSC Force Medical's current process will be phased out.
- 4.1.1. ASR cases should only be active for 3 months, if an applicant cannot complete medical requirements in that time, it should be marked as "paused pending medical requirement needing more than 3 months to complete". These paused cases should be exempted from general statistical reports and instead be listed as their own category. They will also not be viewed as a negative performance metric. The Contractor will be allowed to manage case lifetimes, but MSC reserves the right to require cases to be closed due to excessive amount of time. The general intent is, for applicants that are actively engaging with the Contractor, to resolve their pending medical requirements can be left open and for those who are not to be closed promptly for accurate performance measurement and to not hold applicant non-responsiveness against the Contractor. The 3-month period is a starting point and can be adjusted when sufficient performance data has been collected.
- 4.1.2. Coordination and Communications: The New Hire process of which the creation and submission of the ASR is a critical part of will require that the Contractor coordinate their actions and processes with several MSC departments and other contractors.
- 4.1.2.1. MSC Force Medical: Final authority on medical standards, medical decisions and medical waivers. Responsible for giving CSC the final medical hiring recommendation. Authorizing authority on the usage of the OHS contract.
- 4.1.2.2. MSC N1 and sub codes: Final authority on recruitment tempo, job offers, job re-rates and Reasonable Accommodations (in conjunction with MSC EEO). Generally, the Contractor will communicate with N1 via CSC or Force Medical but may be directed to communicate directly.
- 4.1.2.3. MSC CIVMAR Support Center: Primary customer support point of contact for applicants and go-between the Contractor and N1. They are the authoritative source of applicants for which ASRs are needed and the priority of each applicant. The contractor will need to keep CSC updated on the status of each applicant. CSC and the Contractor will coordinate the tempo and format of status reporting amongst themselves, MSC will serve as a mediator if needed. (CSC is run by a contractor)
- 4.1.2.4. Occupational Health Services Contract: The OHS contract is MSC's primary means to accomplish its occupational health needs. This is run by an OHS contractor who utilizes sub-contracted clinics near the applicant/CIVMAR's home of record or duty location to provide agreed upon medical services. The OHS contract cannot be used for primary care/specialty evaluations of applicants with conditions that require additional medical information/workup or for USCG Medical Exams. The Contractor will need to have lines of communication with the OHS contractor to request the rate required MSP exams and receive their results. The OHS contractor will handle coordination with the applicant to get the MSPs completed.
- 4.1.2.5. Applicant: The Contractor will serve as the conduit between the applicant and MSC Force Medical. Unless requested by MSC Force Medical, applicants should not directly communicate with MSC Force Medical. The Applicant can and for other parts of the New Hire process, must communicate directly with CSC. Applicants generally will not communicate directly with N1. Applicant to N1 communication will typically be routed through CSC unless otherwise directed. After an official request for services has been made to the OHS contractor by CSC or the Contractor, the applicant can then communicate directly with the OHS

contractor. The applicant themselves may not request any new services or change the services requested from the OHS contractor, such requests must come from MSC or its authorized contractors. The applicant may request changes to the date, time and location of services requested from the OHS contractor to meet their needs (within the bounds of the terms of the OHS contract).

4.1.3. Administrative documents. The Contractor will create and/or provide the applicant with the following documents and have applicants sign and submit them to MSC Force Medical as part of the ASR generation process.

4.1.3.1. MSC Privacy Act Statement: Contractor must have applicant sign and retain document on file prior to any medical information being requested or collected. This will be included as a supporting document of the ASR. MSC will provide its Privacy Act Statement for use to the contractor.

4.1.3.2. Chronic Medical Conditions Letter (CMC Letter): MSC Force Medical will create this letter after a FFH determination has been made. Contractor will have applicant review letter and sign it prior to applicant reporting to their scheduled NEO class. Additionally, Contractor will submit signed letter to MSC Force Medical.

4.1.3.3. Permanently Not Fit for Hire Letter (PNFFH): MSC Force Medical will provide this letter to the Contractor when an applicant is deemed PNFFH. The Contractor will give this letter to the applicant and coordinate with the CIVMAR Support Center (CSC) to close-out the application and halt any pending medical actions. MSC Force Medical will directly and simultaneously notify CSC that an applicant has been found PNFFH and is the authoritative source on this status. The Contractor will coordinate with MSC Force Medical in the event the PNFFH determination the applicant wishes to be appeal it. Appeals to PNFFH determinations are handled on a case-by-case basis and may involve other administrative processes within MSC.

4.1.3.4. MSC Medical Waiver: The Contractor will request medical waivers on behalf of the applicant as part of the ASR submission process. If MSC grants the waiver, MSC will create the Waiver letter and send it to the applicant via the Contractor for signature. The Contractor will submit the signed waiver letter back to MSC. No waiver is considered valid until the applicant signed copy is on file with MSC.

4.1.4. ASR Categories:

4.1.4.1. Physically Qualified (PQ)

4.1.4.2. Not Physically Qualified (NPQ)

4.1.4.3. Not Physically Qualified Waiver Requested (NPQ/WR)

4.1.5. ASR requirements:

4.1.5.1. Administrative requirements

4.1.5.1.1. ASR and all supporting documents shall be submitted to MSC via a Contractor provided electronic portal/digital document system. Requirements for this system are outlined in section 8.

4.1.5.1.1.1. File name standard: <LastName, FirstName Middle Initial>-<DOB>-ASR-<ASR category> example: Smith, John A 01Jan1870 ASR PQ

4.1.5.1.1.2. File format shall be a single PDF containing the ASR and all supporting documents apart from the CMC Letter and MSC Medical Waiver Letter which will be separate PDF uploads.

4.1.5.1.2. ASR Format: The ASR itself will be a letter or memo from the Contractor to MSC Force Medical. The layout of and generation method of the ASR will be left to the Contractor, but MSC reserves the right to review and request changes to the layout. The letter will be exportable from the Contractor's data system as a digitally signed PDF.

4.1.5.1.2.1. Minimum required data:

4.1.5.1.2.1.1. Applicant's full name as it appears in MSC's HR data systems and date of Birth

4.1.5.1.2.1.2. Job Rate the applicant is applying for

4.1.5.1.2.1.3. Contractors' recommendation i.e. PQ, NPQ/WR etc.

4.1.5.1.2.1.4. Summary statement of the medical screening results

4.1.5.1.2.1.4.1. List all MSPs and if they meet MSC's standards or what their disqualifying issue is.

4.1.5.1.2.1.4.2. List all medications taken, medical conditions and/or medical history that do not require waivers with a summary, example: "history of childhood asthma, no adult episodes, last episode at age 10, no medications for this issue-NCD".

4.1.5.1.2.1.5. For conditions or medications that require a waiver, a summary statement of the disqualifying condition(s) and results of the medical evaluation and how the condition may meet MSC's waiver standards. Example: "non-insulin dependent diabetes, well controlled on <medication>, per last managing provider evaluation on <date within last 12 months>, current A1c is <X>, meets MSC waiver requirements of well controlled without insulin, A1c below 8%, last evaluation with 12 months-CD/WR". (example provided is not to be viewed as an authoritative listing of diabetes waiver requirements, actual requirements will be found in MSC Force Medical's Waiver Guide)

4.1.5.1.2.1.6. Listing of all providers with their credentials that participated in the gathering/evaluation of the medical data for the ASR. Non-credentialed administrative staff do not need to be listed; it is the responsibility of the credentialed staff to vet their work.

4.1.5.1.2.1.7. The ASR will be signed by the Contractor's Medical Director. The Contractor may be given authorization to give "by direction" authority of this signature to their MD/DOs, PAs or NPs after a satisfactory track record has been established.

4.1.5.2. Medical Screening Requirements: These requirements are subject to change or modification and MSC will keep the Contractor updated. All items mentioned in this section support documents that will be submitted as part of the ASR submission.

4.1.5.2.1. Screening Questionnaire (SQ): MSC will provide its Summary Questionnaire (MSC SQ) to the Contractor to establish the minimum health questions to be answered by the applicant and evaluated by the Contractor. The Contractor may expand on questions asked with prior approval and vetting by MSC. The SQ used by the Contractor will be included in the ASR as a supporting document. The SQ can be provided to the applicant in a manner convenient for the Contractor to collect the information, but the completed

SQ will be provided to MSC as a digital document (PDF) signed by the applicant. A statement that the applicant certifies that their answers are true and complete to the best of their knowledge is required.

- 4.1.5.2.2. USCG Medical Certificate: A Non-Entry Level Med Cert is required for all MSC positions. Applicants who only have an entry level cert should not proceed through the ASR process until they receive their Non-Entry Level Med Cert. The proper cert is completed via a USCG 719K, physical exam form (not USCG 719KE, physical exam form). This is a common error. If the cert states that the applicant has a USCG Waiver, the waiver letter must be provided and included in the ASR, and the condition(s) waived must be evaluated against MSC's standards. MSC has authority to exceed USCG standards and not comply with all USCG Waivers. USCG Med Cert Restrictions are different than waivers but also need to be evaluated against MSC's standards since they may disqualify an applicant from some ratings. Neither MSC nor the Contractor will provide assistance with the performance of USCG exams such as finding a provider to perform the exam or paying for the exam (finding errors and advice on how to correct them is acceptable). This cert is considered a baseline requirement of employment and is the sole responsibility of the applicant. Two of the more common errors with the USCG Med Certs are listed below with the typical solution.
- 4.1.5.2.2.1. No Food Handling: Is a restriction that is often due to the examining provider filling the 719K out incorrectly and applicants should be advised to have this corrected by the provider that did the 719K exam (this may require resubmission to the Coast Guard and the issuance of a new Med Cert and the ASR process should pause until completed). If this restriction is verified as not an error, then this needs to be taken into consideration during the ASR process since it will disqualify them from some rates.
- 4.1.5.2.2.2. Daylight Only Operations: Is a restriction that could result from incomplete color vision testing, it should be verified with the applicant on if they had Color Sense testing done and not just color vision testing (such as PIP or Ishihara). If the applicant had Color Sense testing and failed it, then the restriction is valid and needs to be taken into consideration during the ASR process since it will disqualify them from some rates. If they did not have Color Sense testing done, they should be referred to the provider that did their 719K exam for the Color Sense testing and their 719k updated (this may require resubmission to the Coast Guard and the issuance of a new Med Cert, and the ASR process should pause until completed).
- 4.1.5.2.3. Medical Interview: An interview after the SQ (see 4.1.5.2.1) is completed and valid USCG Med Cert is received is required. This interview can be conducted virtually and must be performed by at least an RN. The purpose of the interview is to get more complete answers to yes questions and to give the applicant a chance to answer questions differently, more completely, or clarify their answers. Applicants will be verbally counseled at the start of the interview that failure to disclose a complete and accurate medical history can result in revocation of the job offer, disciplinary action, and/or termination of employment. MSC will provide its best practices/ guidance, and the Contractor will utilize it to create their interview procedures, which will be reviewed by MSC. The interview notes will be included in the ASR as a supporting document.
- 4.1.5.2.4. Medical Surveillance Exams (MSP): Applicants, based on their job rate, may need up to 4 MSP exams. These exams must be completed and included in the ASR. The exams will be completed via MSC's existing Occupational Health Services contract. The Contractor must take into consideration the applicant's ability to meet their rate's required MSPs. Should an applicant be permanently not qualified for a required MSP by MSC based upon OHS Contractor's findings, they should be found NPQ and not recommended for hire or recommend for a job rate change. This job rate change is at

MSC N1's authority and discretion; CSC will notify the Contractor of the job rate change. If an applicant accepts a job rate change, the ASR needs to be modified to reflect the change or recompleted. The medical standards for these MSPs are set forth in the Navy and Marine Corps Public Health Center Technical Manual NMCPHC-TM OM 6260 Medical Surveillance Procedures Manual and Medical Matrix, however MSC has authority to grant waivers. The OHS Contractor is responsible for ensuring that MSPs conducted meet all technical and administrative requirements and any defects of that nature should be referred to them for correction.

4.1.5.2.4.1. Which MSPs the applicant will need is determined by the MSP Requirements by Rate Chart found in enclosure 2 of N0202H.6110.2-Q CIVMAR MEDICAL AND PHYS STANDARDS. The MSPs are as follows:

4.1.5.2.4.1.1. MSP 503- Hearing Conservation

4.1.5.2.4.1.2. MSP 710- Forklift Operator

4.1.5.2.4.1.3. MSP 721- Explosives Handler

4.1.5.2.4.1.4. MSP 704- Weight Handling Equipment

4.1.5.2.4.2. When an MSP is found to be not within medical standards, a determination needs to be made on whether it is due to a facility error or a medical finding. This may require coordination between the Contractor, the OHS Contractor, and MSC Force Medical. In the case of a facility error such as a missing test result the OHS Contractor is responsible for notifying the facility and getting the correction made. Medical findings are the responsibility of the applicant to resolve, such as failing vision testing due to a lack of eyeglasses or outdated prescription or elevated blood pressure via their personal doctor and health insurance

4.1.5.2.4.3. . For most medical findings a re-examination by the original facility is not required or authorized. The Contractor will work with the applicant to submit medical documentation correcting the findings and then enter an addendum to the MSP stating the issue was corrected, attaching the submitted documentation. In some cases, MSC Force Medical may need to make the decision on clearing an MSP medical finding.

4.1.5.2.5. Small Arms Medical Qualification: Applicants will need to be evaluated in accordance with OPNAVINST 3591.1. SERIES. Applicants with conditions, medications, medical history of concern for Small Arms handling will need to be referred to MSC Force Medical for Small Arms evaluation. MSC Force Medical will provide the Contractor with a specific list of issues that will trigger this evaluation. Force Medical will request additional medical information/evaluations on/from the applicant via the Contractor. The authority to make Small Arms medical qualification determinations and to grant or deny Small Arms waivers rest solely with MSC Force Medical and cannot be delegated. The ASR cannot be completed until after the Small Arms evaluation is completed by MSC Force Medical. Applicants that cannot meet the Small Arms medical requirements may need to have their job offer changed to a new job rate, if they are applying to a rate that requires the handling of Small Arms. This job rate change is at MSC N1's authority and discretion, and CSC will notify the Contractor of the job rate change. If an applicant accepts a job rate change, the ASR needs to be modified to reflect the change since other medical requirements may have changed as well. An applicant's Small Arms medical disqualification, in and of itself, should not result in an NPQ ASR recommendation unless they are denied a rate change or they decline the rate change offer. Applicants that are applying for rates that do not require Small Arms qualifications but are found to be not qualified for Small Arms, can have a PQ ASR, but the non-qualified status must be listed since it will affect what rates they could promote into later. Applicants that do not

require a Small Arms evaluation are to have a statement in the ASR that they meet the requirements to be Small Arms eligible.

- 4.1.5.2.6. Working Aloft Medical Qualification: Applicants that are in Working Aloft required rates need to have their weight screened to ensure they do not exceed the limits for MSC's fall arrest equipment. Applicants that exceed the limits will be NPQ for their rate (but not necessarily NPQ for MSC employment) and a re-rate request will need to be made. The Contractor will, while the ASR is still incomplete send a rerate request via CSC on behalf of the applicant. If the rerate request is declined by N1 or the applicant declines the re-rate offer, the ASR should be closed out as NPQ for MSC employment. Otherwise, the ASR should be recompleted with the new rate's requirements. This job rate change is at MSC N1's authority and discretion and CSC will notify the Contractor of the job rate change. An alternate path if the applicant wants to remain in their current rate, is to request that their ASR be paused to provide time to lose weight and get below the equipment limit, their new weight needs to be documented by a medical professional and not self-certified. The applicant needs to be counseled in writing that they must keep the weight off and that in the event they report to the training center out of standard, they could be required to re-rate, as per N1's rules and policies.
- 4.1.5.2.7. TB/LTBI: All applicants must be screened for TB/LTBI in accordance with MSC Force Medical's TB/LTBI policy prior to completion of the ASR. QuantiFERON Gold (QFT/QFT-G) is MSC's standard for TB testing. Since the QFT is not cross-reactive with the TB BCG vaccine, history of BCG and a positive QFT is not viewed as a false positive and will be treated as evidence of LTBI. If the applicant requires treatment of LTBI, it generally needs to be completed before submitting the ASR, MSC will keep the Contractor informed of exceptions to this requirement. Applicants with active TB must complete treatment prior to submission of the ASR. Due to the low prevalence rate of TB in the US, it is MSC's preference that applicants with a positive QFT be evaluated by their local Public Health Department and not their private physician.
- 4.1.5.2.8. Vaccine Records: The applicant is responsible for providing complete and accurate copies of their vaccine records to MSC via the Contractor. MSC will vaccinate at its expense those who did not provide adequate documentation of MSC required vaccines. MSC will not conduct or assist with titer testing, nor will the Contractor. The applicant may, at their own expense, provide titer testing in lieu of vaccine administration records.
 - 4.1.5.2.8.1. Vaccine Waivers: The Contractor will inform applicants of the vaccine waiver process as early in the ASR process as possible so that waivers can be approved/denied prior to MSC's vaccination event.
 - 4.1.5.2.8.1.1. Medical Waiver: Applicant must submit medical documentation from a board-certified Allergist or Immunologist documenting a medical allergy to a vaccine or vaccine component. This will be submitted as part of the applicant's ASR in the same manner as all other medical waiver requests.
 - 4.1.5.2.8.1.2. Non-Medical Waiver: If the applicant wants a non-medical waiver, they must request it from N1 and MSC EEO via the RA process. The Contractor will coordinate this request with CSC, and the ASR is not dependent on this request being completed but the ASR will note the status of the non-medical waiver request.
- 4.1.5.2.9. Provider Review and Recommendation:
 - 4.1.5.2.9.1. The Contractor will conduct a medical review of all submitted documents and requirements and via the ASR format provide a summary of the applicant's medical history, list all potential disqualifying conditions, state if a MSC waiver is being requested and how the applicant meets the waiver requirements. The Contractor's provider will state if they feel the applicant is PQ, NPQ, or NPQ/WR

to the best of their knowledge. The phrases “fit for hire (FFH)”, “not fit for hire (NFFH)”, “permanently not fit for hire (PNFFH)” and “fit for hire/NEO (FFH/NEO)” are MSC terms of art and will not be used by the Contractor in any communication to an applicant or in an ASR.

4.1.5.2.9.2. The medical interview mentioned in 4.1.4.2.3 can, at the Contractor’s discretion, be conducted as part of the Provider review.

4.1.5.2.9.3. Based on the medical interview and provider review, if additional medical information is needed, the Contractor will request it from the Applicant. If a medical evaluation is needed it will be conducted at the applicant’s expense and cannot be performed or coordinated by the Contractor under this contract. If the Contractor wishes to offer assistance with the additional medical evaluations needed it must be done as a wholly separate process between the applicant and Contractor.

5. Training and Teaming: Due to the critical nature of the New Hire process and the role the ASR will play in it, close coordination between the Contractors staff and MSC Force Medical will be vital to success.

5.1.1.1. Initial contractor training: On initiation of this contract, the Contractor’s team will undergo on-site training with MSC Force Medical’s staff. The length of this training will be negotiated with the Contractor, taking their current experience into account. This training is intended to serve several functions.

5.1.1.1.1. Demonstrate MSC’s current practices and their reasoning to assist the Contractor in building their own practices and provide an initial venue for MSC to review and approve Contractor process that require it.

5.1.1.1.2. Provide a deep understanding of MSC’s medical standards, their importance and reasoning and address the Contractors’ questions or concerns with them.

5.1.1.2. Quarterly provider meeting and training: Will be conducted primarily virtually but can be hosted onsite with MSC or with the Contractor. Quarterly is the intent but can be adjusted as needed. One onsite with MSC meeting per year is required. The primary purpose of these meetings is to resolve issues, set future plans, goals and maintain solid teamwork.

5.1.1.3. Training for new contractors: New staff added by the Contractor in the first year of the contract (after the initial contractor training was completed) will require onsite training with MSC. After the first year of the contract and considering the Contractors’ performance, the FMO may allow the Contractor to bypass this requirement and conduct all training themselves and/or through virtual meetings with MSC Force Medical if they are satisfied with performance. The FMO reserves the right to withdraw this allowance for self-training at any time.

5.1.1.4. All Contractor staff handling medical information will undergo initial HIPAA training and then annual refresher training. The Contractor will provide training that meets the U.S. Department of Health and Human Services Office for Civil Rights standards and present proof of this training upon request.

6. Medical Credentialing:

6.1. License: Any medical personnel (listed in 6.2) assigned by the Contractor to perform the services covered by this contract shall have a current license to practice medicine in any State, Territory, or Commonwealth of the United States or the District of Columbia. All licenses held by medical personnel working on this contract shall be full and unrestricted licenses. Medical personnel who have current, full, and unrestricted licenses in one or more states, but who have, or ever had, a license restricted, suspended, revoked, voluntarily revoked, voluntarily surrendered pending action, or denied upon application will not be acceptable to work on this contract.

6.2. Board Certification and Experience

6.2.1. Nurse

- 6.2.1.1. Registered Nurse or higher. Board certified by the American Association of Occupational Health Nurses (AAOHN) preferred but not required but occupational health experience is highly desired.

6.2.2. Nurse Practitioner

- 6.2.2.1. Occupational Health experience and AAOHN certification preferred, no Nurse Practitioner specific board certification exists.

6.2.3. Physician Assistant

- 6.2.3.1. Occupational Health experience preferred, no specific board certification exists.

6.2.4. Medical Doctor/Doctor of Osteopathy (MD/DO), including the Medical Director

- 6.2.4.1. Board certified in Occupational Medicine by the American Board of Preventative Medicine (ABPM) is required.

6.3. Credentialing and Privileging: Credentialing will be required for any Medical Personnel. The Contractor is responsible for ensuring that the proposed Medical Personnel possess the requisite credentials to grant privileges.

- 6.3.1. No services shall be provided by any medical personnel prior to completing the credentialing process.

- 6.3.2. If medical personnel or other contract provider(s) are not credentialed and privileged or have credentials/privileges suspended or revoked, the Contractor shall furnish an acceptable substitute without any additional cost to the government.

6.4. Technical Proficiency: Medical personnel shall be technically proficient in the skills necessary to fulfill the government's requirements, including the ability to speak, understand, read and write English fluently. The contractor shall provide documents upon request of the KO/COR to verify current and ongoing competency, skills, certification and/or licensure related to providing care, treatment and/or services performed. The contractor shall provide verifiable evidence of all educational and training experiences, including any gaps in educational history for all medical personnel, and the contractor's physician(s) shall be responsible for abiding by the MSC Force Medical's Medical rules, and regulations (referenced herein) that govern medical staff behavior.

7. Governing policies and guidance:

- 7.1. MSC shall provide copies of all pertinent policies and guidance governing the medical standards for applicants to the contractor.

- 7.2. Updates, changes, and deletions shall be provided to the contractor within 5 business days of taking effect.

8. Electronic Portal Requirements: The following features represent MSC's Force Medical's required features/options/workflow(s).

- 8.1. Medical case management software. Intent is for the entire medical screening process to happen and be tracked within a single software/portal environment

- 8.1.1. Ability to have letters/forms auto generated off templates with applicant demographics, editable to meet unique requirements, support digital signature

- 8.2. Each applicant as a case

8.3. Contractor providers/staff as case resources

8.3.1. The Contractor can structure their workflows to suit their needs, but it needs to be fully transparent to MSC

8.4. MSC Workflow

8.4.1. Force Medical staff as case resources

8.4.2. RR case status as standalone work bin

8.4.3. Ready for MSC review as a work bin

8.4.3.1. Force Medical staff members will assign cases in the MSC review bin to MSC providers for review into individual inboxes

8.4.3.1.1. MSC supervisory staff only should have access to move an assigned case to a different MSC provider

8.4.4. Cases tagged/highlighted with priority

8.4.4.1. Expedite, Priority 1, Priority 2, Priority 3

8.4.4.1.1. MSC will provide level criteria to the contractor (typically based on recruitment needs)

8.4.5. Cases auto tagged/highlighted with response time suspense

8.4.5.1. MSC admin/supervisory staff need to have access to set MSC response time requirements to match MSC staffing levels

8.4.5.2. Green-2+ business days remaining

8.4.5.3. Yellow 1 business day remaining

8.4.5.4. Red case has exceeded response time

8.4.5.5. Gray cases paused

8.5. Statistical reporting

8.5.1. All open cases report

8.5.1.1. Submitted weekly to Force Medical and CSC

8.5.1.2. This is a basic weekly status report and can be combined with reports 8.5.2, 8.5.4 if doing so simplifies the data retrieval process so long as the resulting combined report can be easily sorted by data type.

8.5.2. All closed cases report

8.5.2.1. Submitted weekly to Force Medical and CSC

8.5.3. Case report by ASR category

8.5.3.1. Submitted Quarterly and Annually to Force Medical

8.5.3.2. This is a trend analysis report

8.5.4. Paused cases report

8.5.4.1. Submitted weekly to Force Medical and CSC

8.5.4.2. This report is to show what cases have been paused, why and for how long to enable trend analysis

8.5.5. Review Requested case workload report

8.5.5.1. Submitted Monthly to Force Medical

8.5.5.2. This report is intended to be a tool to identify areas of the process that need more clarity so that the Contractor doesn't need to continue requesting guidance for a specific issue

8.5.6. Workload report by case resource

8.5.6.1. Submitted Quarterly to Force Medical along with an annual summary

8.5.6.2. This report needs to break out Contractor case resources (8.3) and MSC case resources (8.4.1) and is intended to verify if the right amount of case resources have been allocated to the right places.

8.5.7. Time to completion graphing/report

8.5.7.1. Submitted Quarterly to Force Medical along with Annual summary

8.5.7.2. This is for trend analysis and Contractor performance report

8.5.8. Contractor desired report(s)

8.5.8.1. As needed

8.5.9. Ad hoc reporting option

8.5.9.1. As requested

9. Phase in Phase out

9.1. The Contractor shall provide a phase-in plan specifying how they will ensure personnel and procedures will be in place to accomplish the complete scope of work within 10 business days after the contract award. Within 3 days after submitting this plan, the contractor will schedule a kick-off meeting with the COR and GOV POC's to discuss this phase-in plan.

9.2. There will be a Contract Phase-Out Period commencing thirty (30) days prior to end of contract. The purpose of the Phase-Out Period is to ensure a smooth transition of services from the predecessor contract to the new contract. Upon notification of the government, the Contractor shall provide the services necessary to ensure a successful transition of services from it to the awardee under a subsequent contract for the same or similar requirement. No later than ninety (90) days prior to the end of the contract, the Contractor shall develop, with input from the government, a plan of action to facilitate the transition of services to the incoming Contractor with no degradation in services.

PART 10

TECHNICAL EXHIBIT LISTING

10.1 LIST OF TECHNICAL EXHIBITS:

- (1) Performance Requirements Summary
- (2) Deliverables Schedule

DRAFT

TECHNICAL EXHIBIT 1

PERFORMANCE REQUIREMENTS SUMMARY (PRS)

Failure to meet a Performance Requirement Summary metric is considered a major nonconformance that adversely (or potentially) impacts mission, safety of personnel and/or equipment, environment, performance (quality), schedule (delivery), and/or cost. The government's contracting officer will communicate major non-conformances to the contractor on a Contract Discrepancy Report (CDR) form with a suspense date for the contractor's corrective action plan.

Note: Government surveillance of contractor performance is not limited to the Performance Objectives as outlined in this PRS. The government reserves the right to conduct compliance surveillance of any contractual requirement for this acquisition.

Performance Objective	Standard	Performance Threshold	Method of Surveillance	Consequence
PRS # 1. The Contractor shall provide Applicant services as required by the contract.	The Contractor provided Applicant services should be completed in accordance with the delivery requirements required by the PWS.	There should be no deviation from the requirements of the PWS.	COR Tracking and Complaints received from the customers. Random sampling may occur at the discretion of the COR.	Negative CPARS Rating / Deduction and/or CDR
PRS # 2 The Contractor shall provide deliverables accurately and by the times specified in the contract.	Deliverables are correct and on time when submitted.	Deliverables should be reviewed and delivered with zero errors.	COR and/or TCOR review and customer complaints. Random sampling may occur at the discretion of the COR.	Negative CPARS Rating / Deduction and/or CDR
PRS # 3 The Contractor shall provide ASRs for all applicants to MSC.	ASRs are correct and within required times as specified in the PWS.	The ASRs should be reviewed and delivered with zero errors.	100% inspection by.	Negative CPARS Rating / Deduction and/or CDR
PRS #4 The Contractor is responsible for its employees completing and staying up to date on all training required by MSC. Some examples of required training are: HIPAA.	The Contractor shall provide documentation to the COR evidencing that Contractor employees have completed the required training.	Within five (5) days of the specific training due date.	100% inspection by the COR.	Negative CPARS Rating / Deduction and/or CDR

TECHNICAL EXHIBIT 2

DELIVERABLES SCHEDULE

<u>Deliverable</u>	<u>Frequency</u>	<u>Submit To</u>	<u>CDRL</u>
Medical Staff Credential Report	Within 60 Days after Contract Award and Quarterly Updates thereafter	COR	TBD
Contractor Personnel Security Report	As Required	COR	TBD
Safety Plan	Within 30 Days after Contract Award	COR	TBD
Mission Continuity Plan	Within 90 Days after Contract Award	COR	TBD
Disaster Recovery Plan	Within 90 Days after Contract Award	COR	TBD
Certificate of Insurance (COI)	15 days after Contract Award	COR	TBD
Phase-In Plan	Within 5 Working Days after Contract Award	COR	TBD
ASR	Submitted to with 3 working days of completion	Force Medical	TBD
Open Case Report	Weekly	Force Medical and CSC/N1	TBD
Closed Case Report	Weekly	Force Medical and CSC/N1	TBD
Case report by ASR category	Quarterly report and Yearly report	COR	TBD
Paused cases report	Weekly	Force Medical and CSC/N1	TBD
Review Requested case report	Weekly	COR	TBD
Workload report by MSC case resource	Quarterly report and Yearly report	COR	TBD
Time to completion report	Quarterly report and Yearly report	COR	TBD